



SEXUAL HEALTH EDUCATION IN CANADA

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SEXUAL HEALTH EDUCATION IN CANADA

- In Canada, education (including sexual health education) is the responsibility of the **provinces/territories**, resulting in variation across the country
- **The BC curriculum is arguably the most comprehensive**, and has been in place for 30+ years (1995)
- Sexual health education is **mandated** in BC, but the delivery is still “hit or miss”
- Pre-service learning for teachers **does not include training to deliver sexual health education**
- **Gatekeepers:**
 - School boards
 - Principals
 - Community/parents
 - Teachers



BC SEXUAL HEALTH LEARNING STANDARDS:

Elementary School level

Kindergarten-Grade 3 (3 sessions each): *(30min classes for Grades K-2 and 45min classes for Grade 3)*

- Names for parts of the body, including male and female private parts
- Appropriate and inappropriate ways of being touched
- Hazards and potentially unsafe situations
- Strategies and skills to use in potentially hazardous, unsafe, or abusive situations
- Factors that influence self-identity

Grade 4 and 5 (2-3 sessions each):

- Physical, emotional, and social changes that occur during puberty, including those involving sexuality and sexual identity
- Factors that influence self-identity, including body image and social media

Grade 6 and 7 (2-3 sessions each):

- Physical, emotional, and social changes that occur during puberty and adolescence
- Influences on individual identity, including sexual identity, gender, values, beliefs and relationships
- Practices that reduce the risk of contracting sexually transmitted infections and life-threatening communicable diseases
- Human reproduction (from Science curriculum, but necessary to discuss safer sex practices)



BC Sexual health learning standards (CONT'd):

Secondary School level

Grade 8 and 9 (2-3 sessions each):

- Influences of physical, emotional, and social changes on identities and relationships
- Consequences of bullying, stereotyping, and discrimination
- Sources of health information
- Potential short-term and long-term consequences of health decisions, including protection from sexually transmitted infections
- Healthy sexual decision-making

BC Prescribed Learning Outcomes for Grades 10-12 (1-3 sessions each):

- Demonstrate an understanding of the skills needed to build and maintain healthy relationships
- Analyze practices that promote healthy sexual decision-making
- Analyze practices associated with the prevention of HIV/AIDS



CORE PRINCIPLES OF COMPREHENSIVE SEX ED:

- 1. It is accessible to all people regardless of age, gender, sexual orientation, STBI status, geographic location, socio-economic status, cultural or religious background, ability, or housing status (e.g., those who are incarcerated, homeless, or living in care facilities).**
- 2. It promotes human rights including autonomous decision-making and respect for the rights of others.**
- 3. It is scientifically accurate and uses evidence-based teaching methods.**
- 4. It is broadly-based in scope and depth and addresses a range of topics relevant to sexual health and well-being.**



CORE PRINCIPLES OF COMPREHENSIVE SEX ED (CONT'D):

- 5. It is inclusive of the identities and lived experiences of lesbian, gay, bisexual, transgender, queer, intersex, Two Spirit (LGBTQI2S+), and asexual people.**
- 6. It promotes gender equality and the prevention of sexual and gender-based violence.**
- 7. It incorporates a balanced approach to sexual health promotion that includes the positive aspects of sexuality and relationships, as well as the prevention of sexual health problems.**
- 8. Is responsive to and incorporates emerging issues related to sexual health and well-being.**
- 9. Is provided by educators who have the knowledge and skills to deliver comprehensive sexual health education and who receive administrative support.**



U.S. FEDERAL DEFINITION OF “ABSTINENCE EDUCATION”



Under Title V, Section 510 of the 1996 Social Security Act, P.L. 104–193, the term “abstinence education” is defined as an educational or motivational program which [1]

- (A) has as its exclusive purpose, teaching the social, psychological, and health gains to be realized by abstaining from sexual activity
- (B) teaches abstinence from sexual activity outside marriage as the expected standard for all school-aged children
- (C) teaches that abstinence from sexual activity is the only certain way to avoid out-of-wedlock pregnancy, sexually transmitted diseases, and other associated health problems
- (D) teaches that a mutually faithful monogamous relationship in the context of marriage is the expected standard of human sexual activity
- (E) teaches that sexual activity outside of the context of marriage is likely to have harmful psychological and physical effects
- (F) teaches that bearing children out-of-wedlock is likely to have harmful consequences for the child, the child’s parents, and society
- (G) teaches young people how to reject sexual advances and how alcohol and drug use increases vulnerability to sexual advances
- (H) teaches the importance of attaining self-sufficiency before engaging in sexual activity

IMPACT OF AOUM PROGRAMS

Adolescence is marked by the emergence of human sexuality, sexual identity, and the initiation of intimate relations; within this context, abstinence from sexual intercourse can be a healthy choice. However, programs that promote abstinence-only-until-marriage (AOUM) or sexual risk avoidance are **scientifically and ethically problematic** and as such have been widely rejected by medical and public health professionals. Although abstinence is theoretically effective, in actual practice, intentions to abstain from sexual activity often fail. Given a rising age at first marriage around the world, a rapidly declining percentage of young people remain abstinent until marriage. ... The weight of scientific evidence finds that AOUM programs are **not effective** in delaying initiation of sexual intercourse or changing other sexual risk behaviors. AOUM programs ... inherently withhold information about human sexuality and may provide medically inaccurate and stigmatizing information. Thus, **AOUM programs threaten fundamental human rights to health, information, and life.**

Young people need access to accurate and comprehensive sexual health information to protect their health and lives.

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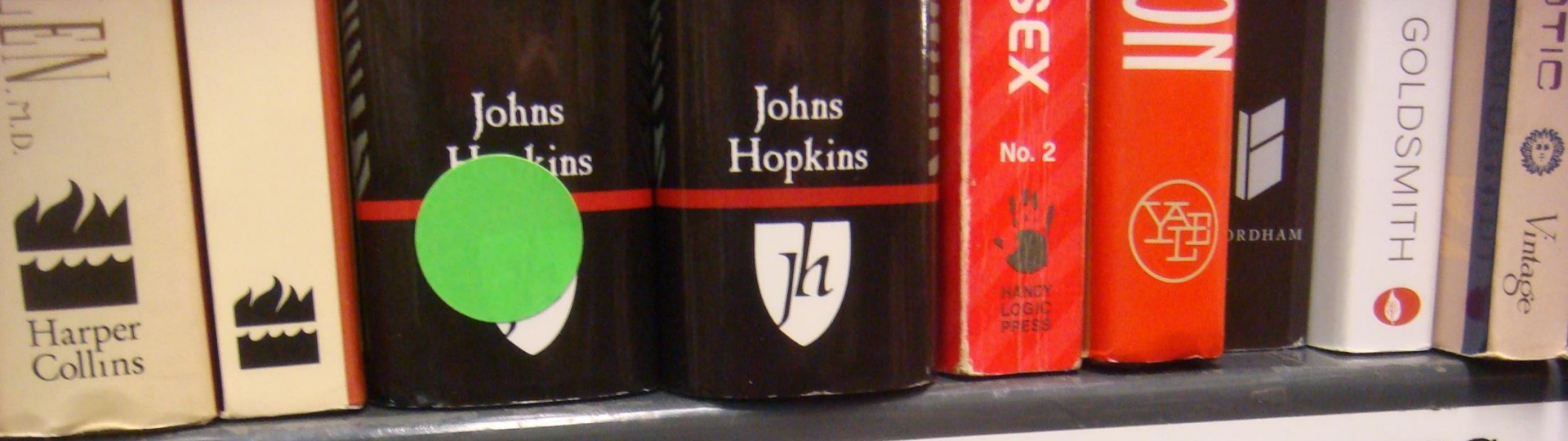


MEANWHILE, IN HOLLAND...

A woman with short brown hair, wearing a brown blazer over a grey top, stands in the foreground and speaks. She is gesturing with her hands. In the background, several young people are sitting on a concrete ledge in an outdoor setting with a modern building and trees. The scene is slightly out of focus behind her.

Caroline de
Cristofaro

In the Netherlands, we offer children
and young adults



SEXUALITY CONTINUES



THE UNIVERSITY OF BRITISH COLUMBIA

